

How to Open a Non-Medical Home Care Agency in Arizona

A step-by-step guide to starting a personal care, attendant care, and homemaker agency — in the one state that does not license it

PolicyReady.org — Practical Playbooks for Care Entrepreneurs

The Arizona difference — read this first

Arizona does not license non-medical home care. There is no state license for personal care, attendant care, homemaker, or companion services — only skilled medical home health (nursing, therapy) is licensed, by the Arizona Department of Health Services. That makes Arizona the lowest-barrier state in the country to open a home care agency. But “no license” does not mean “no rules.” This guide walks you through exactly what you DO have to do: screen your caregivers through the Arizona Fingerprint Clearance Card and the abuse registries, meet the Direct Care Worker training and testing standard, and — if you want Medicaid-funded clients — enroll with AHCCCS and contract with the ALTCS health plans. Do those well and you can open faster here than almost anywhere.

How to Use This Guide

This guide takes you from idea to first client. Read it once start to finish, then use it as a checklist. Everything here is general information for prospective Arizona agency owners — not legal, tax, or billing advice. Requirements, fees, and phone numbers change, so verify each step with the agency named before you rely on it. Where this guide gives a dollar figure or a code, treat it as a starting point to confirm, not a final answer.

The one-paragraph version

Form an Arizona LLC and get an EIN. Decide your service lines (personal care, attendant care, homemaker, companion) and your payers (private pay first, Medicaid/ALTCS later). Put your insurance, bond, and workers' comp in place. Build your caregiver pipeline: every caregiver needs an Arizona Level One Fingerprint Clearance Card, an APS and DCS registry check, and — for Medicaid work — Direct Care Worker training and testing plus CPR and first aid. Name a supervisor who assesses clients, writes service plans, and does supervisory visits at least every 90 days. Start with private-pay clients while you enroll as an AHCCCS provider through APEP and contract with the ALTCS health plans. Capture EVV on every Medicaid visit. Then grow through referral relationships with hospitals, skilled nursing facilities, hospices, and senior communities.

The Path at a Glance

Ten steps take you from idea to an operating Arizona non-medical home care agency:

1. **Decide your model.** Private pay only, or private pay plus Medicaid/ALTCS? Which services, which service area?
2. **Form the business.** Register an LLC or corporation with the Arizona Corporation Commission; get an EIN; open a business bank account.
3. **Put protection in place.** General and professional liability insurance, a surety or fidelity bond, and workers' compensation coverage.
4. **Write your policies.** Adopt a policy and procedure manual, client forms, and a caregiver handbook — your operating standard in place of a license.

5. **Build caregiver screening.** Fingerprint Clearance Card, APS and DCS registry checks, and exclusion screening for every hire.
6. **Set up training.** Direct Care Worker training and testing, CPR, and first aid — required for Medicaid work, best practice for all.
7. **Name a supervisor.** A qualified person to assess clients, write service plans, and do supervisory visits at least every 90 days.
8. **Enroll to bill Medicaid.** Register as an AHCCCS provider through APEP and contract with the ALTCS Program Contractors; set up EVV.
9. **Start serving clients.** Begin with private pay while enrollment is pending; deliver, document, and supervise every visit.
10. **Grow through referrals.** Build relationships with hospitals, skilled nursing, hospices, and senior communities in your metro.

Part 1 — What This Business Is

A non-medical home care agency sends caregivers into clients' homes to help with everyday life: bathing, dressing, grooming, toileting, transferring, and feeding (personal care and attendant care); cleaning, laundry, meals, and errands (homemaker services); and supervision and companionship (companion services). It is not skilled care — no nursing, no wound care, no medication administration, no therapy. Those services require a licensed home health agency and a nurse. Your caregivers help people stay safe and independent at home; they do not treat illness.

Arizona is a strong market for this business. It has one of the fastest-growing older populations in the country, huge retirement communities across the Valley and in Tucson and Prescott, and a steady stream of “snowbirds” who need seasonal help. Families overwhelmingly prefer to keep aging parents at home, and both private pay and Arizona’s Medicaid long-term care program fund in-home care. Demand is real and growing.

The service lines, in plain terms

- **Personal care / attendant care:** hands-on help with the activities of daily living. In Arizona’s Medicaid program, attendant care combines personal care, homemaking, and general supervision, and can even go with the member outside the home.
- **Homemaker:** housekeeping, laundry, meal preparation, shopping, and errands that keep a home safe and livable — provided in the client’s own home.
- **Companion:** supervision, socialization, and support with no hands-on personal care — good for clients who mainly need presence and safety.
- **Respite:** short-term relief for a family caregiver — often the entry point for a new client relationship.

Why Arizona, specifically

Three features make Arizona unusually favorable for a new agency. First, the demographics: the state’s 65-and-over population is large and growing fast, concentrated in places built for retirees — the West Valley communities around Sun City and Surprise, the East Valley (Mesa, Gilbert, Chandler), the Tucson and Green Valley areas, and the Prescott “mile-high” region. Second, the seasonal “snowbird” population swells each winter with older adults who arrive without their usual support network and need help fast. Third — and this is the decisive one — Arizona’s open-entry rule means you can be serving clients while an agency in a licensed state would still be waiting on a survey. The combination of high demand and low barrier is rare.

It is worth being clear-eyed about the flip side. Open entry means competition: because anyone can open, your market has many small agencies, and families and referral sources have choices. You do not win on being allowed to operate — everyone is allowed. You win on trust: rigorous screening, real supervision, reliable caregivers who show up, and clean documentation. That is why this guide spends so much time on the parts that are optional in the sense that no surveyor enforces them, but essential in the sense that they are your entire competitive advantage.

The economics, roughly

The core economics of a home care agency are simple: you bill a client or payer an hourly rate, you pay your caregiver an hourly wage, and the spread covers your overhead (screening, training, supervision, insurance, bond, office, scheduling, billing) and your profit. In Arizona your wage floor is the state minimum wage, but competitive caregiver pay usually runs above it, and you must budget for overtime after 40 hours, paid sick time, and workers’ compensation. Private-pay rates are set by you and the market; Medicaid rates are set by the member’s assessment and your health-plan contract. Most agencies find private pay carries a healthier margin per hour, while Medicaid brings volume and steadiness once you are enrolled and contracted. A common path is to start private-pay for cash flow and margin, then add ALTCS to fill the schedule.

Part 2 — The No-License Advantage, and What You Still Must Do

Here is the single most important thing to understand about opening in Arizona: the state does not license non-medical home care. The Arizona Department of Health Services licenses skilled home health agencies and hospices, but personal care, attendant care, homemaker, and companion services fall outside that. There is no application to ADHS, no survey to pass, and no license fee for a non-medical agency. Entry is open.

That is a genuine advantage — you can be operating in weeks, not months — but it puts the burden of quality on you. Nobody from the state is going to inspect you into competence. The agencies that win in an open market are the ones that operate to a high standard anyway, because clients, families, hospital discharge planners, and health plans can tell the difference. So while there is no license, there is a real list of things you must do:

- **Screen every caregiver.** An Arizona Level One Fingerprint Clearance Card, plus separate Adult Protective Services and Department of Child Safety registry checks. Anyone barred from a card or with a substantiated abuse finding cannot work with your clients.
- **Train to the Direct Care Worker standard.** To serve Medicaid members, your caregivers pass Arizona's Direct Care Worker competency tests within 90 days of hire, and hold CPR and first aid. It is the right bar to hold even for private-pay caregivers.
- **Enroll and contract for Medicaid.** If you want ALTCS-funded clients, you enroll with AHCCCS and contract with the ALTCS health plans, and follow their supervision, EVV, and billing rules.
- **Meet federal and employment law.** HIPAA privacy, OSHA safety, the FLSA home care rule (overtime, no companionship exemption for agencies), Arizona's minimum wage and paid sick time, and workers' comp.
- **Report abuse.** Every staff member is a mandatory reporter to Arizona Adult Protective Services.

The mindset that wins in an open-entry state

Operate as if you were licensed. Adopt a real policy manual, screen and train rigorously, supervise every case, and document everything. When a hospital discharge planner or an ALTCS case manager is deciding which agency to trust, the one that looks and runs like it is surveyed wins the referral — every time.

Part 3 — Screening & Caregiver Qualifications

Your caregivers are your business. In a state with no license, how you screen and train them is your entire quality story. Get this right and everything else follows.

The Fingerprint Clearance Card

Every caregiver must hold a valid Arizona Level One Fingerprint Clearance Card, issued by the Arizona Department of Public Safety after a fingerprint-based state and federal criminal background check. The caregiver applies through the DPS Public Services Portal, gets fingerprinted, and receives a card that is valid for six years. You keep a copy in the personnel file and track the expiration so it never lapses.

The abuse registries — checked separately

The Fingerprint Clearance Card covers the criminal background check, but it is not enough on its own. You must also check each caregiver against the Adult Protective Services registry (for substantiated findings of abuse, neglect, or exploitation of a vulnerable adult) and the Department of Child Safety registry. A caregiver with a substantiated finding on either registry is barred from working with your clients. Run both checks before the caregiver ever meets a client, and document the results.

A practical note on sequencing: because the Fingerprint Clearance Card can take a few weeks to arrive, start the application the moment you make a conditional offer, and use the wait to complete the registry checks, references, and health screening. Keep a simple tracker for every caregiver showing card status and expiration, registry-check dates, exclusion-screening dates, and training status — so you can prove, at a glance, that no one is with a client before screening is complete. Build the rule into your scheduling: an unscreened caregiver simply cannot be assigned.

Exclusion screening

Separate from the criminal and registry checks, screen every caregiver, contractor, and vendor against the federal exclusion lists (the OIG List of Excluded Individuals/Entities and the SAM exclusions) and the AHCCCS exclusion list — before hire and again every month. Employing or paying an excluded person for work connected to Medicaid-funded services is a serious problem that can trigger repayment and penalties. This takes minutes with the online tools and protects you from an expensive mistake.

Direct Care Worker training and testing

To provide attendant care, personal care, or homemaker services to Medicaid (ALTCS) members, a caregiver must pass Arizona's standardized Direct Care Worker competency tests within 90 days of hire. The curriculum is the Principles of Caregiving — a Fundamentals module plus an Aging and Physical Disabilities module — delivered and tested through an AHCCCS Approved Training and Testing Program. You can become an approved program yourself or contract with one. Registered nurses, LPNs, certified nursing assistants, and approved assisted-living caregivers are exempt from the DCW test. Testing records transfer between employers, so a caregiver who tested elsewhere usually does not retest unless they have been out of the field for about two years. Every caregiver also holds current CPR and first aid, and completes the annual continuing education and fraud-waste-and-abuse training AHCCCS requires.

The DCW test has two parts — a written test and a hands-on skills test — covering the fundamentals of caregiving (communication, infection control, safety, body mechanics, assisting with the activities of daily living) plus content specific to aging and physical disabilities. Deciding whether to become your own Approved Training and Testing Program is a real strategic choice: doing it in-house costs setup effort but lets you train, test, and onboard caregivers on your own schedule, which is a genuine advantage in a tight labor market. Many new agencies start by contracting with an existing approved program and bring it in-house once they have volume.

Even if you never touch Medicaid

Hold the DCW bar anyway. If you serve only private-pay clients, you are not legally required to run the DCW training

and testing. But it is the recognized Arizona competency standard, and training your caregivers to it — and documenting it — is one of the cheapest ways to look and perform like the best agency in your market.

Part 4 — Setting Up to Operate

Form the business

Register an LLC or corporation with the Arizona Corporation Commission, get a federal EIN, and open a business bank account. Obtain any city transaction-privilege or business license your location requires. There is no ADHS home care license to add — this is the full extent of your “licensing” for a private-pay non-medical agency.

Insurance, bonding, and workers’ comp

Carry general and professional liability insurance, and a surety or fidelity bond (or third-party crime insurance) that protects clients against loss from caregiver theft — essential when your workers are alone in clients’ homes and sometimes handle their money. Arizona requires workers’ compensation coverage for your employees. Confirm all of it is active before any caregiver starts.

Policies, a supervisor, and service plans

Adopt a policy and procedure manual, client intake and service forms, and a caregiver handbook. Name a qualified supervisor — for many agencies a nurse or an experienced care manager — who assesses each client, writes a written service plan, reviews it with the assigned caregiver, and conducts supervisory visits as often as needed and at least every 90 days for ALTCS members. This supervision is your quality-control system; treat it as non-negotiable. Keep a complete, confidential record for every client and every caregiver.

Part 5 — Finding Clients & Referrals

Two engines fill your schedule: families who pay privately, and the aging-services network that steers people to agencies. Build both.

- **Hospital discharge planners and case managers** send people home who need help — be the agency they trust to prevent a readmission.
- **Skilled nursing and rehab facilities** discharge clients who still need in-home support after therapy ends.
- **Hospices** refer both ways: they need aides for personal care, and you refer declining clients to them.
- **Assisted living and senior communities** have residents who need one-on-one help beyond what the community provides.
- **Elder-law attorneys and geriatric care managers** guide families to trustworthy agencies and often manage ALTCS planning.
- **The public aging network** — the Area Agencies on Aging and the Eldercare Locator (1-800-677-1116) — routes families to services.

Your PolicyReady Referral System for Arizona lists named hospitals, skilled nursing facilities, hospices, senior communities, funeral homes, elder-law attorneys, and care managers across metro Phoenix, Tucson, the East Valley, and northern Arizona, with the eleven-part playbook for turning them into a steady stream of clients.

A word on how referral relationships actually work

Referral sources send clients to agencies they trust to make them look good. A discharge planner who sends you a patient is putting their own reputation on the line — if your caregiver is late, unprepared, or unreliable, it reflects on them. So the way you earn referrals is not a sales pitch; it is a track record. Take the first few referrals from a source and execute flawlessly: answer the phone, start on time, communicate, and never leave a shift uncovered. Report back to the source that the client is doing well. Do that a handful of times and you become the agency they call first. This is slow at the start and then compounds — which is exactly why building it early matters.

Building & Keeping Your Caregiver Team

In home care, your caregivers are the product. A family never sees your office; they see the person who walks through their door. Recruiting, screening, training, and keeping good caregivers is therefore the operational heart of the business — and in a tight labor market, retention is as important as recruiting.

- **Recruit continuously.** You will always need caregivers; treat recruiting as an always-on activity, not a reaction to a staffing gap.
- **Screen without shortcuts.** The Fingerprint Clearance Card, registry checks, and exclusion screening are your promise to clients — and your legal footing. Never bend them to fill a shift.
- **Onboard well.** Orient every caregiver to your policies, the non-medical scope, client rights, abuse reporting, and the specific service plan for each client before the first visit.
- **Supervise for real.** Supervisory visits are not paperwork; they catch problems early, keep caregivers supported, and are your quality signal to payers.
- **Retain by respect.** Reliable scheduling, fair pay at or above minimum wage, prompt paychecks, and being treated well keep caregivers — and caregiver turnover is the biggest hidden cost in this business.

Part 6 — Getting Paid

Non-medical home care in Arizona is paid three main ways: privately by the client or family; by Arizona's Medicaid long-term care program (ALTCS); and by other sources such as long-term-care insurance and Veterans benefits. Most new agencies start with private pay — it needs no enrollment — and add Medicaid once they are established.

Private pay

Set a clear hourly rate, put it in a written service agreement, and bill only for services actually delivered. Private pay is the fastest way to open: no enrollment, no contracting, no EVV — just good care and honest billing.

Medicaid — AHCCCS and ALTCS

Arizona's Medicaid program is AHCCCS. Its long-term care arm, the Arizona Long Term Care System (ALTCS), funds attendant care, personal care, homemaker, and respite for eligible elderly and physically disabled members. To bill it, you (1) enroll as an AHCCCS provider through the AHCCCS Provider Enrollment Portal (APEP), paying the enrollment fee and passing a background screening, and (2) contract with the ALTCS Program Contractors — the managed-care health plans (such as Banner University Family Care, Mercy Care, and UnitedHealthcare Community Plan; verify the current plans) — whose members you want to serve. Enrolling does not by itself bring clients; the contracts with the health plans are what let you serve their members. Services follow each member's Person-Centered Service Plan, written with an ALTCS case manager. You revalidate your AHCCCS enrollment about every four years.

The enrollment sequence, in order, looks like this: obtain a National Provider Identifier for the agency; create your APEP account and complete the provider-enrollment application for the right provider type; pay the application fee (a CMS-set amount, around \$750 in recent years — confirm the current figure); complete the required fingerprint-based background screening and any site visit; receive your AHCCCS provider ID; then approach each ALTCS health plan to request a provider agreement. The health-plan contracting step is the one new owners underestimate — it is a separate process with each plan, and it is what actually connects you to members. Plan for the whole path to take weeks to months, and keep serving private-pay clients while it runs.

Member-directed options

Arizona lets ALTCS members direct their own care through two models you should understand, because they change your role. Under Agency with Choice, the member helps recruit and direct a caregiver while your agency remains the legal employer (handling payroll, screening, and oversight). Under Self-Directed Attendant Care, the member takes on more of the employer role. Knowing which model a member uses tells you what your agency is responsible for, and both are common in Arizona's system.

Electronic Visit Verification (EVV)

For Medicaid-funded attendant care, personal care, homemaker, and respite, Arizona requires Electronic Visit Verification — an electronic record of each visit's service, client, caregiver, date, location, and start and end times. You use the AHCCCS EVV system or an AHCCCS-approved alternate, and your EVV data must match what you bill. Get EVV working before your first Medicaid visit; EVV problems delay or deny payment.

Service (billing) codes

Arizona's ALTCS services use standard national procedure codes, but AHCCCS and each health plan assign the exact code, modifier, and rate for your contract, and rates are set by the member's assessment — there is no single published rate. Confirm every code and rate with AHCCCS and your Contractor before you bill. As a starting reference:

Service	Common code (per 15 min unit)	What it covers
Personal care	T1019	Hands-on help with the activities of daily living

Service	Common code (per 15 min unit)	What it covers
Attendant care	S5125	Personal care + homemaking + general supervision (Arizona's combined service)
Homemaker	S5130	Cleaning, laundry, meals, shopping in the client's home
Companion	S5135	Supervision and socialization, no hands-on care
Respite (unskilled)	S5150	Short-term relief for a family caregiver

Codes shown are common national HCPCS references for these services; AHCCCS and your ALTCS Contractor assign the exact code, modifier, and rate. Verify before billing.

Other funding sources

Source	Who it helps	What to know
Private pay	Families who can self-fund	Fastest to start; no enrollment; set a clear written rate
ALTCS (Medicaid)	Financially and functionally eligible seniors and adults with disabilities	Requires AHCCCS enrollment + health-plan contract + EVV; served under a PCSP
Long-term-care insurance	Policyholders	Agency bills the policy or the family submits; confirm the policy's home-care terms
Veterans benefits	Wartime veterans and surviving spouses	VA Aid & Attendance can fund in-home care; an accredited VA agent can help families qualify

A word on wages and overtime

Arizona's minimum wage is \$15.15 an hour in 2026 (higher in Tucson and Flagstaff, and adjusted each year for inflation). It applies no matter how small your agency is. There is no Arizona overtime law, so the federal FLSA governs: pay overtime after 40 hours in a week, and remember that as a third-party employer you cannot claim the "companionship" exemption. Build these costs into your rates from day one, and add Arizona's earned paid sick time and workers' comp.

Part 7 — Home, Heat & Safety

Care happens in the client's home, so home safety is part of the job. Your supervisor assesses each home for fall risks, fire and electrical hazards, unsafe equipment, and conditions that make safe care hard, and reflects what they find in the service plan. Caregivers report new hazards they see.

Arizona's signature risk: extreme heat

Heat kills homebound seniors in Arizona every summer. Build heat safety into your care: check that clients have working cooling and cold water, teach caregivers to recognize heat exhaustion and heat stroke, and escalate immediately when a home is dangerously hot or the air conditioning fails. Your emergency plan should also cover monsoon storms and flash flooding, wildfire and smoke, dust storms, and power outages — and identify which clients are most at risk so you can reach them first.

Part 8 — Your First 90 Days

1. **Days 1–30:** Form the LLC, get your EIN and bank account, and bind insurance, bond, and workers' comp. Adopt your policy manual, forms, and handbook.
2. **Days 1–30:** Stand up caregiver screening — Fingerprint Clearance Card, APS and DCS registry checks, exclusion screening — and your DCW training/testing path.
3. **Days 30–60:** Name your supervisor and finalize assessment and service-plan templates. Hire and fully screen your first caregivers. Take your first private-pay clients.
4. **Days 30–60:** Start your AHCCCS enrollment through APEP and reach out to the ALTCS health plans about contracting; set up your EVV system.
5. **Days 60–90:** Run real supervisory visits, tighten documentation, and begin referral outreach to two or three hospitals, skilled nursing facilities, or hospices in your metro.

Part 9 — Common Mistakes to Avoid

- **Assuming “no license” means “no rules.”** The screening, training, Medicaid, employment, and reporting requirements are all real — skipping them is how open-entry agencies get into trouble.
- **Letting a caregiver start before screening is complete.** No Fingerprint Clearance Card or registry check means no client contact. No exceptions.
- **Skipping supervisory visits.** Missed 90-day visits are both a compliance failure for ALTCS and a quality risk in every case.
- **Thinking AHCCCS enrollment brings clients.** You still have to contract with the health plans and build referral relationships.
- **Sloppy EVV or documentation.** If it is not verified and documented, Medicaid will not pay it — and an audit will find it.
- **Underpricing.** Build wage, overtime, sick time, workers' comp, insurance, and supervision into your rate from the start.
- **Providing skilled care.** The moment a task needs a nurse, it is outside your scope — refer it out.

Part 10 — Staying Compliant

In a state with no license to renew, compliance is a set of ongoing habits you own. Nobody sends you a renewal notice, so you build your own calendar and work it. The recurring obligations to track are:

- **Per caregiver, ongoing:** Fingerprint Clearance Card expiration (six years), CPR and first aid renewals, and annual continuing education and fraud-waste-and-abuse training.
- **Monthly:** re-run exclusion screening on all staff and vendors; review open incidents and complaints.
- **Every 90 days:** supervisory visits for every active ALTCS client (and on your own schedule for private-pay clients).
- **About every four years:** revalidate your AHCCCS provider enrollment through APEP.
- **Annually or as due:** insurance and bond renewals, workers' comp, corporate and tax filings with the state, and any health-plan re-credentialing.

Run a simple quality program alongside the calendar: track complaints, incidents, missed visits, supervisory findings, EVV compliance, and client satisfaction, look for patterns, and fix what they show. In an open-entry state your quality program is the internal substitute for the survey you never get — it is what holds you to a high standard when no regulator does.

The bottom line

Arizona lets you open faster than anywhere — so open well. Screen every caregiver, train to the Direct Care Worker standard, supervise every case, enroll cleanly if you want Medicaid, capture EVV, document honestly, and build referral relationships one flawless client at a time. The absence of a license is not permission to cut corners; it is an invitation to compete on quality — and quality is what turns a new agency into the one a hospital, a hospice, and a family call first.

Two Worked Examples

Example 1 — A private-pay start in metro Phoenix

You form an LLC, bind insurance and a bond, and adopt your policy manual. You hire two caregivers, put them through Fingerprint Clearance Cards, APS/DCS registry checks, and DCW-level training with CPR and first aid, and name yourself supervisor. Your first client comes from a hospital discharge planner who needs safe help for a patient going home after a fall. You assess the home, write a service plan, and start daily personal care — private pay, billed weekly. As you prove reliable, that discharge planner sends more. Referral targets you would build in Phoenix include:

- **Hospitals:** Banner – University Medical Center Phoenix (602-839-2000); Dignity Health St. Joseph's Hospital & Medical Center (602-406-3000)
- **Skilled nursing / rehab:** Desert Peak Care Center (602-243-6121); Northpark Health & Rehabilitation (623-248-5696)
- **Hospices:** Hospice of the Valley (602-530-6900); United Hospice & Palliative Care of Arizona (602-296-4717)
- **Assisted living / senior communities:** Clarendale Arcadia (480-573-3700); The Manor Village at Desert Ridge (480-764-3793)
- **Elder-law / care managers:** Emily R. Taylor, Attorney PLLC (480-699-3145); Senior Care Management Solutions, LLC (480-502-2273)

Example 2 — Adding Medicaid in Tucson

Your Tucson agency has a steady private-pay base and you are ready for ALTCS. You enroll as an AHCCCS provider through APEP, contract with an ALTCS health plan, and set up EVV. A case manager authorizes attendant care for a member under a Person-Centered Service Plan; you assign a DCW who has passed the competency test, capture EVV on every visit, bill the right code, and supervise at least every 90 days. As you take Medicaid clients, you also keep the private-pay side growing through the same referral network. Referral and coordination partners you would work in Tucson include:

- **Hospitals:** Banner – University Medical Center Tucson (520-694-0111); Tucson Medical Center (520-327-5461)
- **Skilled nursing / rehab:** Sabino Canyon Rehabilitation & Care Center (520-722-5515); Catalina Post-Acute & Rehabilitation (520-795-9574)
- **Hospices:** Agape Hospice (520-207-5817); Soulistic Hospice (520-398-2333)
- **Assisted living:** The Ranch Estates of Tucson (520-277-9388); The Fountains at La Cholla (833-409-9349)
- **Elder-law / ALTCS planning:** Fleming & Curti PLC (520-622-0400); Paul B. Bartlett, P.C. (520-750-1061)

Beyond the two big metros

The same playbook works in Arizona's other markets. In the fast-growing East Valley (Mesa, Gilbert, Chandler), your first referral relationships might include hospitals such as Dignity Health Mercy Gilbert Medical Center (480-728-8000) and Banner Gateway Medical Center (480-543-2000), skilled nursing partners like Santé of Mesa (480-699-9624), and senior communities such as Pebble Ranch Senior Living (480-977-2937). In the Prescott and northern Arizona region, you would build relationships with Dignity Health Yavapai Regional Medical Center East (928-445-2700), hospices like Arizona Life Hospice – Yavapai (928-379-3391), and assisted-living communities such as Touchmark at The Ranch (928-632-7800). Your Arizona Referral System lists named partners across all four regions.

Part 11 — Growing From Here

Once you are steady, growth comes from three moves: deepen referral relationships until you are the agency a hospital or hospice calls first; add payers (layer ALTCS onto private pay, then long-term-care insurance and VA benefits); and add capacity by becoming your own AHCCCS Approved Training and Testing Program so you can

train and onboard caregivers faster than competitors. Keep your quality visible — in an open-entry state, reputation is the whole game.

Who to Call & What to Ask

For...	Who to contact
Forming your business	Arizona Corporation Commission (azcc.gov). Ask: what do I file to register my LLC, and what are the annual requirements?
Fingerprint Clearance Cards	Arizona Department of Public Safety, Public Services Portal (azdps.gov). Ask: how does a caregiver apply for a Level One card, and how long does it take?
Abuse registry checks	Arizona Adult Protective Services registry and DCS Central Registry. Ask: how do I request registry checks on a prospective caregiver?
DCW training & testing	AHCCCS Direct Care Worker helpdesk — dcw@azahcccs.gov or 602-417-4401. Ask: how do I become or use an Approved Training and Testing Program?
Becoming a Medicaid provider	AHCCCS Provider Enrollment (APEP), azahcccs.gov. Ask: how do I enroll as an attendant/personal care provider, and what is the current fee?
Getting ALTCS clients	The ALTCS Program Contractors (health plans). Ask: how do I contract to serve your members, and what are your EVV and billing requirements?
Reporting abuse	Arizona Adult Protective Services — 1-877-767-2385 (SOS-ADULT); DCS — 1-888-767-2445; 911 for emergencies.
Wage & hour rules	Industrial Commission of Arizona (ica.az.gov) and U.S. Dept of Labor. Ask: what are the current minimum wage, paid sick time, and FLSA home care overtime rules?
Finding the aging network	Eldercare Locator — 1-800-677-1116 — connects you to the Area Agency on Aging for your county.

Key Terms

- **Non-medical home care:** personal care, attendant care, homemaker, and companion services — not licensed by Arizona.
- **Level One Fingerprint Clearance Card:** the DPS card, from a fingerprint background check, that every caregiver must hold.
- **APS / DCS registry check:** separate checks for substantiated abuse findings — required in addition to the card.
- **Direct Care Worker (DCW) training & testing:** Arizona's competency standard (Principles of Caregiving) for Medicaid-funded caregivers.
- **AHCCCS:** Arizona's Medicaid program.
- **ALTCS:** the Arizona Long Term Care System — Medicaid long-term care that funds in-home attendant/personal care.
- **APEP:** the AHCCCS Provider Enrollment Portal, where you enroll and revalidate as a Medicaid provider.
- **Person-Centered Service Plan (PCSP):** the ALTCS plan, written with a case manager, that authorizes a member's services.
- **EVV:** Electronic Visit Verification — the electronic record of each Medicaid-funded visit.

Core Rules & References

- A.A.C. R9-10 Article 12 — ADHS home health agency licensing (skilled care only; non-medical home care is NOT licensed)
- A.R.S. § 46-459 & § 8-804 — the APS and DCS abuse registries used to screen caregivers
- A.R.S. Title 41, Ch. 12 — Arizona Level One Fingerprint Clearance Cards (Department of Public Safety)

- AHCCCS AMPM Policy 1240-A — attendant care, personal care, homemaker, and supervision standards
- AHCCCS ACOM Policy 429 — Direct Care Worker training and testing requirements
- 21st Century Cures Act § 12006 — the federal EVV mandate (AHCCCS EVV in Arizona)
- A.R.S. § 46-454 — mandatory reporting of vulnerable-adult abuse to Arizona APS
- The Fair Labor Standards Act home care rule; the Arizona Minimum Wage Act; Arizona's Fair Wages and Healthy Families Act (paid sick time)

This guide is a general educational resource for prospective Arizona non-medical home care agency owners. It is not legal, tax, or billing advice, and requirements, fees, and contacts change. Verify every step, figure, and phone number with the agency named — the Arizona Corporation Commission, the Department of Public Safety, AHCCCS, and the relevant payer — before you rely on it. Provisions cited reflect Arizona law and AHCCCS policy at the time of writing.